

It's Not Your Fault

A Practical Guide for Families Caring for an Aging Parent

Rob Brizzi, CDP · Cardinal Promise Press · 228 pp · 29 chapters · 8.5×11 full color

Where the book is. The manuscript is complete, page-numbered, and near-final — it has been through a structured beta round and several revision passes. I'm looking for an editor's eye before it goes to proof and print.

First thing I'd value from you: tell me what level of edit this actually needs — a light developmental pass, a line edit, a copyedit, a proofread, or some combination. My guess is line + copy, but you'll know better once you're in it.

1 Read in this order

- **Front matter first** — *Why I Wrote This Toolkit, You Are Not Alone, How to Use This Handbook, and Start Here*. This is where the voice and the promise are set.
 - **One full care chapter** — e.g. Ch 2 (Falls) or Ch 12 (Hospice) — to feel the chapter template (30-second box → sections → worksheet → Rob's Note → takeaway).
 - **The end-of-life chapters** — Ch 27–29 — the most sensitive writing in the book.
 - Then the rest at your pace.
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2 What to look at, by pass

Developmental / structural. Does the wayfinding (Start Here, five pathways, the decision tree, the color-urgency tags) genuinely help, or stack too many routing devices up front? Is the five-part order right? Gaps or redundancy across 29 chapters? Does the emotional thread (You Are Not Alone, Rob's Notes, Cardinal Moments) land without tipping maudlin?

Line / voice. Is the voice consistent chapter to chapter — warm, direct, second person, myth-busting? Sentence-level clarity, especially in the money and legal chapters (Parts 3–4). Is the closing refrain "*The next right question is...*" effective, or does it wear out? (It's deliberate — tell me if it's too much.)

Copyedit. Consistency of program names and terms (Medicare/Medicaid parts, POLST/MOST/DNR, "home health" vs "private duty"), hyphenation, numerals, capitalization of the box labels; American English; every acronym defined on first use; cross-references ("see Chapter X") correct.

Proofread (last). Typos, box/label consistency, TOC vs the printed folios (now aligned), index accuracy, widows/orphans.

3 Please give these specific spots your eye

Recently added material — check it blends seamlessly in voice and doesn't repeat what's nearby: the front-matter page *You Are Not Alone; The Fall That Changes Everything* (Ch 2); *You Cannot Save Someone by Destroying Yourself* (Ch 4); and three Family Meeting sections in Ch 5 (*The Conversation Nobody Wants to Start, When One Child Does Everything, What If No One Agrees?*).

"Cardinal Moment" reflections — one-page composite stories closing a chapter, labeled "names and details changed." Only **four** exist so far (Ch 1, 2, 4, 5). Key question for you: should this device be *all-or-nothing* across the book, or is a handful of them the right touch? And does the composite framing read honestly?

- **Care-option chapters (Ch 6–12)** share a parallel structure (what it is / what it isn't / when it helps). Intentional scaffolding, or does it read repetitive? Your call.
- **The Praise page was removed** (endorsements are on hold), so the book currently opens straight into the title page. Fine, or do you want a placeholder?
- **Accuracy & safety.** The book is explicitly educational, not advice, and sends readers back to their own clinicians. It has *not* had a formal clinical review — flag anything that reads as a medical directive rather than "ask your doctor about..."

4 Voice & house style (so edits protect the book)

- American English; second person; plain, compassionate, myth-busting.
- Keep the signature devices intact: the *30 Seconds* box, *Rob's Note*, the *Chapter Takeaway*, the color-urgency tags, and the worksheets/scripts.
- Suggest a light **style sheet** (Chicago is my assumption) with a project term list for the Medicare/Medicaid/legal vocabulary — happy to adopt whatever you recommend.

5 Out of scope for you

Production is handled separately — please don't spend time on the ISBN, page-number folios, cover art resolution, file specs, or distribution. If you spot something there, just flag it in a note.

Let's agree on: the level of edit; deliverable format (tracked changes in Word + a style sheet + a short editorial letter is ideal); the style guide; timeline; and fee/scope. I can send the manuscript as a numbered PDF now, and a Word file for tracked changes if that's how you prefer to work.